Restraint / Restrictive Practice Policy

Policy Title: Restraint / Restrictive Practice

Effective Date: [Insert Date]

Review Date: [Insert Date – usually annually]

Version: 1.0

Applies to: All staff, agency workers, contractors, volunteers, and residents at [INSERT CARE HOME NAME]

1. Policy Statement

At [INSERT CARE HOME NAME], we are committed to safeguarding residents' rights, dignity, and autonomy. We recognise that restraint or restrictive practices should only ever be used as a last resort, when it is necessary, proportionate, and in the best interests of the resident.

We aim to:

- Minimise and, where possible, eliminate the use of restraint.
- Ensure any restrictive practice is legally and ethically justified.
- Protect residents from harm while upholding their human rights.
- Comply with the Mental Capacity Act 2005 (MCA), Deprivation of Liberty Safeguards (DoLS), and Human Rights Act 1998.

2. Purpose

This policy provides a framework to:

- Define restraint and restrictive practices.
- Guide staff on lawful and safe use of restraint.
- Ensure appropriate authorisation, recording, and monitoring.
- Promote alternatives to restrictive interventions.

3. Scope

This policy applies to all staff and covers:

- Physical Restraint – using physical force to control or restrict movement.
- Mechanical Restraint – using devices such as belts or bedrails.
- Chemical Restraint – using medication to control behaviour rather than treat a medical condition.
- Environmental Restraint – restricting movement by controlling access (e.g., locked doors).

4. Roles and Responsibilities

Registered Manager:

- Ensures compliance with legislation and CQC standards.
- Provides staff with training and guidance on restraint and alternatives.
- Reviews incidents, audits practice, and reports concerns.

Senior Staff / Nurses:

- Assess residents' capacity and best interests in line with the MCA.
- Ensure DoLS authorisation is obtained where required.
- Supervise and support staff in decision-making.

All Staff:

- Follow de-escalation and person-centred care approaches before considering restraint.
- Only use restraint if necessary, proportionate, and in line with training.
- Report and record all incidents immediately.

5. Principles for the Use of Restraint

- Last Resort – used only when less restrictive options have failed.
- Proportionality – the minimum necessary intervention for the shortest possible time.
- Best Interests – restraint must be in the resident's best interests under the MCA.
- Least Restrictive Option – always consider alternatives first.
- Dignity and Respect – residents must be treated with compassion and sensitivity.

6. Authorisation and Legal Framework

- Any use of restraint must comply with the Mental Capacity Act 2005 and the Human Rights Act 1998.
- If ongoing restrictions are required, a DoLS authorisation (or Liberty Protection Safeguards when enacted) must be sought.
- Chemical restraint must only be prescribed and reviewed by a qualified healthcare professional.
- Families and representatives must be informed unless this places the resident or others at greater risk.

7. Incident Response and Recording

Any use of restraint must be:

- Recorded in the resident's care plan and daily notes.
- Reported to the manager immediately.
- Reviewed to assess causes, effectiveness, and alternatives.

Serious incidents may need to be reported to:

- The CQC.
- The Local Safeguarding Adults Board.
- The Commissioning Authority.

8. Alternatives to Restrictive Practices

- De-escalation techniques (calm communication, reassurance).
- Environmental adaptations (safe spaces, noise reduction, personalisation).
- Distraction and engagement (activities, sensory interventions).
- Care planning to reduce triggers for distress or behaviour of concern.

9. Monitoring and Review

- All incidents of restraint will be monitored and analysed to identify patterns.
- Resident care plans reviewed regularly to reduce reliance on restrictions.
- Policy effectiveness reviewed through audits, incident analysis, and feedback.

10. Training

- All staff receive mandatory training in:
    - Mental Capacity Act and DoLS.
    - De-escalation and behaviour support.
    - Safe and lawful use of restraint (where appropriate).
- Refresher training provided annually or as needed.

11. References

- Mental Capacity Act 2005
- Deprivation of Liberty Safeguards (DoLS) – DoH Guidance
- Human Rights Act 1998
- CQC Fundamental Standards – Regulation 13 (Safeguarding service users from abuse and improper treatment)
- NICE Guideline NG10: Violence and aggression – short-term management

12. Related Policies

- Mental Capacity Act & DoLS Policy
- Safeguarding Adults Policy
- Risk Assessment Policy
- Incident Reporting Policy
- Mental Health & Wellbeing Policy
- Equality, Diversity & Inclusion Policy

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